Circumcision as a critical test case for the value of religious bioethics: a close examination of the writings of Rabbi Elliot N. Dorff

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Abstract

This essay examines the role of religion in bioethics through the lens of male circumcision, focusing on the work of Rabbi Elliot N. Dorff, a prominent religious bioethicist. The chapter analyses Dorff's distinctively religious approach to bioethics, grounded in the Jewish principle that "God owns the body" (GOTB). While this precept leads Dorff to generally oppose non-voluntary, non-therapeutic body modifications, it concomitantly suggests an exception for penile circumcision, justified on religious grounds. The presence of this exception creates tensions with Dorff's broader ethical framework, particularly regarding bodily integrity and autonomy.

The chapter also explores the implications of Dorff's stance on other forms of child genital cutting, such as those practiced by some Islamic communities (e.g., "ritual pricking" of the vulva of young girls). The authors contend that Dorff faces a dilemma: opposing such practices would conflict with his commitment to religious pluralism, while supporting them would contradict his own religious precepts. How a distinctively religious approach to bioethics can best handle such a dilemma remains an open question.

Ultimately, the authors call for a more robust engagement from religious bioethicists like Dorff on contentious issues such as child genital cutting. They argue that the topic serves as a crucial test case for religious ethics in bioethics, challenging thinkers to apply their principles consistently across different practices and belief systems.

Keywords: Elliot Dorff, circumcision, ritual pricking, bodily integrity, religious bioethics, children's rights

Introduction

Should religion be permitted a seat at the table in bioethics? For this chapter, we have been asked to consider how the topic of circumcision might inform a response.

The simple answer is: Yes, of course, religion (and especially religious individuals) should have a seat at the table of bioethics, where bioethics is understood to be a deliberative, theoretical and practical inquiry into the normative dimensions of biomedicine, biotechnology, and related areas. Since religious people, practices, and beliefs are often implicated in these normative dimensions (the case of circumcision, being sometimes a religious and sometimes a medical procedure, is a good example of this; see Reis 2021), it is both necessary and desirable for bioethics and bioethicists to engage directly with the topic of religion, and with religious people.

Of course, there are different views about how best to "do" bioethics, or what rules should govern bioethical debate and deliberation, whether about religious topics or seemingly more secular ones. But whatever these "rules of engagement" are, or should be, we would simply maintain that they must apply equally to all participants. So, while religion (and especially religious individuals) should certainly have a seat at the table of bioethics, the seat should be of the same general size and make as any other. In short, there should be no *special* seats for religious bioethicists, or for non-religious ones, or anyone else.

In all cases, debaters should be clear about their premises and show how their conclusions follow. And if they aim to be persuasive, it will usually be prudent to pick premises that are widely shared among the interlocutors. Alternatively, it may be useful to show how *different* premises can -- when combined with certain plausible auxiliary assumptions -- get you to the same destination. Anyone can make a good or a bad argument. So, for example, secular thinkers sometimes propose weak or objectionable arguments about matters of religion or bioethics, and religious thinkers sometimes propose strong and compelling ones. Other times, it's the other way around. And sometimes, as we'll see, religious and non-religious philosophers reach similar conclusions about what should be done, or what some policy should be, albeit via different pathways.

As one of us has argued, "we all have certain meta-ethical commitments (whether implicit or explicit, religious or otherwise) and we all have to try to convince those we disagree with that our meta-ethical commitments make more sense than theirs do, or do a better job of explaining a shared moral intuition, or whatever. That's just 'doing philosophy'" (Earp 2015, p.

865; responding to Biggar 2015). In other words, we *all* should strive to come to the table with clear argumentation, consistent positions, and a readiness to receive and respond to critique.

In this chapter, we will examine the issue of nontherapeutic childhood circumcision (medically unnecessary removal of part or all of the penile foreskin of a child who is too young to consent) from a philosophical and bioethical perspective. Predominately, we will be concerned with the notion of a child's right to bodily integrity (RBI) and how this might best be understood.

When writing in our own voice, we will approach the topic from a secular bioethical perspective: that is, we will not appeal to any particular (i.e., particular to some community rather than widely shared) religious or metaphysical viewpoint to justify claims or arguments. In other words, we will do our best to proceed on the basis of public reason (Rawls 1997; Van Howe 2013a). However, it will also be important to know how a religious bioethicist might think about the notion of a child's RBI in relation to circumcision. For it would shed light on a deeper question, namely: In the absence of medical necessity, when, if ever, are the religious beliefs or motivations of a child's parents or guardians sufficient to justify *any kind* of surgical intervention into a child's sexual anatomy (Van Howe 2013b; Sarajlic 2014; Möller 2017; Chambers 2018; for an alternative perspective, see Jacobs & Arora 2015)?

As a test case for this line of inquiry, our focus will be on the writings and speeches of noted bioethicist Rabbi Elliot N. Dorff, to see how his distinctively *religious* approach to bioethics might contribute to longstanding debates about child genital cutting. We have chosen to focus on Rabbi Dorff because, as we are tasked with exploring the relevance of religion to bioethics, few better exemplify the intersection.

As a rabbi and Jewish educator, Dorff has been hailed as the foremost bioethicist of the Jewish Conservative movement, and a guiding figure of Jewish bioethics writ large. Yet, Dorff has equally been sought by important secular ethics groups and initiatives (indeed, up to the national level) to provide an authoritative religious perspective on a breadth of moral issues. Of course, his positions, like the positions of any rabbi or educator, cannot be read as *the* definitive characterization of Jewish ethics (especially as regards the particular character of the Jewish religion, which is an interpretive tradition). However, he has often been viewed by secular bodies interested in incorporating religious perspectives into their work as a leading authority on Jewish religious law. According to Louis E. Newman, founding president of The Society of Jewish Ethics:

> Elliot's reputation extends far beyond the Jewish community. His standing as one of the country's leading Jewish ethicists has earned him invitations to address several national

> commissions and bioethics task forces, including the National Bioethics Advisory Commission, where he has testified on human cloning and stem cell research; the Surgeon General's Commission on Responsible Sexual Behavior; and the National Human Resources Protections Advisory Commission. He also served, in 1993, on the Ethics Committee of Hillary Rodham Clinton's Health Care Task Force. In addition, Elliot has participated in several other collaborative projects through the American Association for the Advancement of Science and the Carnegie Foundation. In all these contexts, Elliot offers a distinctively Jewish voice, yet one capable of speaking to a diverse audience. And in each of these settings, he has earned a reputation for the depth of his learning, the respectful way he listens to others, and the incisive way he frames the key issues. It is no wonder that whenever people are looking for a Jewish authority on major moral issues, Elliot's name rises to the top of the list. (Newman and Dorff 2024, p. xiii)

Indeed, speaking generally, we, too, regard Dorff as a worthy illustration of how a religious ethicist can fruitfully participate in, and contribute to, bioethics as a discipline. He comes to the "table" with thoroughly researched and well-argued positions, and a wealth of knowledge about the strengths and weaknesses of competing moral systems as they relate to contemporary issues. Importantly, Dorff is also not shy about critiquing his own religious tradition; addressing moral pitfalls of various religious practices; reassessing positions based on new information; and identifying areas for—and pathways toward—moral progress within his religious community. The potential for such progress, as we will show below, may be attributed in large part to Dorff's high valuation of vigorous debate.

So, what is his position on circumcision? Seemingly, this would be a central issue on which Dorff, given his influence, approach, and specialized training, would be especially well-suited to contribute. After all, circumcision of the penis—a matter of particular importance in traditional Jewish practice while also being customary in Islam and incidentally practiced within some Christian communities—falls directly at the intersection of religion, biomedicine, health, and sexuality.

Curiously however, we are left to speculate on Dorff's precise conception of the ethics of circumcision and of the child's right to bodily integrity (RBI), as he has said surprisingly little about either. For example, in Dorff's chapter on "Beginning of Life" issues in *The Oxford Handbook of Jewish Ethics and Morality* (2016), although he takes positions on multiple relevant issues including abortion and reproductive rights, genetic testing, sexual health, and more, there is not a single mention of circumcision. The subject is likewise absent (mentioned only in

passing, and not engaged with as a bioethical concern) from Dorff's seminal three-volume series, *Matters of Life and Death: a Jewish approach to modern medical ethics (1998), To Do the Right and the Good: a Jewish approach to modern social ethics (2002),* and, *To Love Your Neighbor and Yourself: a Jewish approach to modern personal ethics (2003),* which are considered by many as canonical works of contemporary Jewish ethics.

This is, in our view, a striking omission, not only due to the significance of circumcision in traditional Judaism, but also due to its being a bioethically contested US American cultural practice (Bedzow 2020). Without knowing Dorff's position on this critical issue, or how it relates to his position on other, seemingly analogous practices, such as "ritual pricking" of the vulva for religious reasons (discussed below), we may be missing a crucial insight into the role of religion in bioethical debate. In what follows, then, we take a deeper look at the nature and strength of Rabbi Dorff's distinctively religious approach to bioethics, hoping that this may serve as a window into the value of religious approaches to bioethics more generally.

Framing our discussion: the right to bodily integrity

Opponents of non-therapeutic, non-voluntary circumcision of the penis (NNC, we'll say) typically argue that the practice violates—or at least, infringes—the child's right to bodily integrity (RBI) (e.g., Van Howe et al. 1999). How exactly this right should be formulated is a matter of ongoing debate (Mazor 2013; Viens 2017; Townsend 2021; Earp 2022a; Pugh 2023; Mazor 2024), but arguments in this vein typically highlight the following features of NNC which it shares with other genital cutting practices widely viewed as problematic:

(a) It targets, and concentrates surgical risk on, the sexual anatomy of a non-consenting person;
(b) It removes healthy, sensitive tissue from -- and thus permanently alters -- a person's genitals (i.e., their so-called "private" parts) without their permission;
(c) It does these things without medical necessity

On some views, then, *whatever* the child's RBI consists of, it is the sort of thing that is violated -- or at least, infringed[1] -- by any practice that has these features (BCBI 2019, 2024;

[1] Philosopher Joseph Mazor, for example, suggests that the child's RBI may permissibly be infringed upon (and therefore not actually violated) in cases when a competing interest or other right may be argued to take priority (see Mazor 2019).

see also Higashi 2023). In addition to so-called “normalization” surgeries on the genitals of infants born with intersex traits, which are becoming increasingly controversial (Carpenter 2018; Ziemińska 2020; Muschialli et al. 2024), the most obvious set of practices that share these features with NNC are the ones that fall under heading of ‘female genital mutilation’ or ‘FGM’ as defined by the World Health Organization (WHO): for example, what is called FGM Type 1a (cutting or removal of tissue from the clitoral hood, leaving the labia and clitoral glans/body intact); FGM Type 2a (cutting or removal of tissue from the inner labia, without cutting or modifying other parts of the vulva); or FGM Type 4, which “includes all other harmful procedures to the female genitalia for non-medical purposes, e.g., pricking, piercing, incising, scraping and cauterizing the genital area” (WHO 2023).

Importantly, the WHO definition and typology for ‘FGM’ does not apply solely to highly invasive forms of ritualistic cutting or modification of the vulva, such as infibulation, but also to forms that are, on any view, much *less* invasive than NNC of the penis, and which are, moreover, often done in the same families for similar reasons (O’Neill, et al. 2020; Shweder 2022; Buckler 2024; Earp et al. 2024). Thus, it includes a practice known in English as ‘ritual pricking’ (*khatna* in Arabic; FGM Type 1a or 4 on the WHO typology), in which a ceremonial drop of blood is drawn from a young girl’s vulva, as we explore later in the chapter. According to the WHO, *all* such female genital practices, being both medically unnecessary and non-consensual, violate the child’s RBI—that is, *irrespective* of the anticipated harm-level, *irrespective* of parental motivation, *irrespective* of whether the practice has been medicalized, and *irrespective* of any claimed benefits, whether physical or psychosocial (Van Eekert et al. 2024). If that is correct, it might seem to follow that NNC must likewise violate a child’s RBI, *whatever* that right consists of, because it appears to meet the same basic criteria (Davis 2003, Van Howe 2011).

Of course, not everyone agrees with the WHO’s position on ‘ritual pricking’—much less the proposed extension of this position to NNC. As proponents of (or defenders of toleration for) ritual pricking argue, the child’s RBI is not, in fact, violated by the practice, because it is insufficiently *harmful* (Arora & Jacobs 2016; Duivenbode & Padela 2019; Porat 2021; Shweder 2022). Setting aside how ‘harm’ is to be understood in such cases (Van Howe 2011; Earp and Darby 2017), such an analysis implies that a child’s RBI is only violated by practices that, perhaps among other things, meet some minimal threshold of harmfulness. More generally, then, it is ostensibly an *empirical* matter (once we agree what harm is, how to measure it, and what the actual threshold should be) whether any given practice is in fact harmful enough to violate a child’s RBI.

On the other hand, the RBI might be an *autonomy*-based right, or at least a right whose 'job' is to protect (future) autonomous decision-making (Earp 2019; Bagattini 2015; Munzer 2018; Pugh 2023). Thus, it might entail that certain types of decisions—for example, decisions about whether non-therapeutic surgical risk should be concentrated on one's sexual anatomy—should be left to the affected individual to make, if and when they are able to make it. According to this interpretation, if a person is *capable* of consenting to circumcision or ritual pricking, but fails to do so, the right is violated by the practice, *irrespective* of any anticipated empirical consequences. And similarly, this view suggests, if a person is *not (yet) capable* of consenting to a medically unnecessary genital cutting procedure, to impose the practice anyway would be to violate their *future* right to decide for themselves (i.e., it would violate the right in advance) (Munzer 2018; BCBI 2024; see also Darby 2013).

We will not aim to settle such matters in this chapter (though we have argued elsewhere for something like the latter position: see Buckler 2023; Earp and Steinfeld 2018; Earp 2019). Our point is only that the bioethicist whose work we are discussing, Rabbi Elliot N. Dorff, has not clarified his position either way.

Suppose he *does* think NNC (and perhaps also ritual pricking) infringes the child's RBI, but that the state should permit such infringement if it is done for religious reasons. If so, there are various ways to make such an argument (e.g., Jacobs & Arora 2017), and it would be useful to see them made. On the other hand, suppose he does *not* think NNC infringes, much less violates, the child's RBI. Well, then, it might be that he accepts a broadly harm-based formulation of the right (along with the necessary conceptual and empirical premises). Or perhaps he thinks that *ritual pricking* violates the right, but NNC does not—a possibility we explore below. Either way, it would be illuminating to know: What is Rabbi Dorff's position?

The answer to this question matters. And it is a key example of why we need religious bioethicists "at the table" to think our way through it. Suppose we are responsible for setting up a taskforce to determine policy recommendations related to the child's RBI. Our job might be to determine under what, if any, conditions, a clinician should be permitted to cut a small child's genitalia for non-therapeutic reasons. Now, who might we select for such a group? Would we benefit from Rabbi Dorff's perspective?

Dorff's Jewish ethics

Although Dorff does not explicitly write about NNC or ritual pricking in his works on ethics, we do have some clues about his general approach to conceptualizing bodily integrity. Dorff commonly describes the core foundation of Jewish bioethics as tied to the theological conception that "God

owns the body" (hereafter, "GOTB"). Here is how Dorff explains this precept in his chapter on "Beginning of Life" issues in *The Oxford Handbook*:

> A number of Judaism's fundamental convictions affect Jewish approaches to the moral issues at the beginning of life.
>
> 1. The body belongs to God. Unlike American secular ethics, in which each person's body belongs to him or herself, Jewish classical texts assert that God, as Creator of the universe, owns everything in it, including our bodies. Therefore God can and does make certain demands of us as to how we use our bodies, demands articulated in Jewish law. It is as if we were renting an apartment: we have fair use of the apartment during our lease—and, in the biological analogue, during our lease on life—but we do not have the right to destroy the apartment (commit suicide) or harm it unnecessarily, because it is not ours. (Dorff and Crane 2013, p. 313)

Now, secular liberal ethicists may balk at this suggestion as it appears on its face to be, far from a common starting point for discussion, a maximally dogmatic illiberal creed. For one, it implies that it is morally appropriate for humans to be treated as property (albeit belonging to a transcendent owner), and thus—on Dorff's theological explanation—not entitled to independent, individual rights over their own bodies.[2] The specifics of God's "demands" on us may also be disturbing, as many are manifestly illiberal and at odds with widespread contemporary ethical norms. But perhaps even more affronting, to the secularist, is the implication that this principle applies not only as a guide for how individuals should treat their own bodies, but also in terms of how empowered agents (for example parents and medical professionals) ought to treat vulnerable individuals at the beginning of life (such as infants and small children) who may or may not grow up to share their overarching metaphysic (Earp 2022b).

However, it is crucial to understand that Dorff is not a religious fundamentalist or legal positivist. He does not argue that the biblical commandments—no matter how explicitly stated in the text—are beyond moral evolution through human interpretation and critique. Indeed, as Dorff's co-editor of the *Oxford Handbook*, and scholar of Jewish ethics, Jonathan K. Crane, notes:

[2] Dorff suggests that Jewish ethics operates through enforcement of *duties* (or commandments) rather than *rights*. He claims that this is a valuable mode through which to pursue ethics as it adds a active rather than passive impetus to behave morally (Newman and Dorff 2024, see section beginning on page 31).

> One could take Dorff to task, [by] saying that he (sometimes) holds a wrong faith.... Dorff evades this critique by turning not to claims of belief or theology but to hermeneutics. Hence he argues that norms bequeathed to the living must be continuously and communally re-evaluated in light of contemporary circumstances and sensibilities and thus avoid becoming "wrong" in the strong sense. This process therefore juxtaposes moral sensibilities and the Judaic textual tradition because, according to Dorff, they are neither coextensive nor coterminous. (Crane 2009, p. 151)

Accordingly, Dorff acknowledges, "with the possible exception of some right-wing Orthodox groups, all modern Jews see the world through Enlightenment glasses, where the individual is the fundamental reality" (Dorff 2007, p. 33). Dorff also strongly defends the separation of church and state. For example, he has said that "Jewish law, [however] interpreted, should not be the basis of American law and neither should the stance of any other religion" (Congregation Brith Shalom 2022). And so, Dorff cedes priority to civil law to govern over all religions (more on this below).

One of the most helpful explanations Dorff offers for engagement with his work is a reference he makes to Thomas Jefferson's conception of "the public square," in which Dorff claims, "the model is that you come together to determine public policy bringing with you your convictions, whatever they are. And the presumption is that through debate, very vigorous debate, that you would ultimately come to a much wiser public policy than if people came to that discussion pretending that they didn't have [their personal] convictions" (Glazer Institute 2013). Therefore, upon our assessment, Dorff uses the principle of GOTB, not as a dogma, but more so as a particularistic "framework" or perhaps a "state of mind" which he brings to ethics discourse. He uses this framework to argue for a religiously grounded general ethic but without an invincible conviction that any specific religious moral or traditional practice will remain accepted merely on the merit that his religion commands it.

This explains why Dorff rarely if ever uses the theological notion of GOTB to defend controversial scriptural edicts in the modern context. Much to the contrary, Dorff's positions have a marked tendency to align with the liberal mainstream, leading Dorff to take opposing conclusions to many of his contemporaries in Orthodox branches of Judaism. For example, Dorff interprets the Jewish religious tradition as functionally compatible with reproductive freedom even though many Orthodox authorities disagree (Dorff and Crane 2016; Eisenberg 2004). Dorff supports gender equality in civil and religious matters despite thousands of years of codified religious patriarchy in Judaism (Kimmel 2021). He supports sexual equality/freedom, and opposes domestic violence

(Dorff 1995), even though the bible explicitly condones many actions which would qualify as domestic violence under today's standards (Karman 2022).

More recently, Dorff has reassessed one of his previously most conservative positions, on so-called 'medical aid in dying' (MAID); while formerly categorically opposed, he has softened over the last few decades, in line with advancements in civil policy and pressure from religious peers (Dorff 2020). There is perhaps no better example of Dorff's predilection for liberalism than his co-authoring of an influential 2006 *responsum* (rabbinic ruling) which permits the ordination of openly gay clergy (Dorff 2006). In spite of the seemingly unambiguous decree in Leviticus 20:13 that (male/male) gay sex is a sin punishable by death, Dorff argues that homosexual Jews should not only be openly integrated within the Jewish community, but elevated to the highest positions of communal authority.

Thus, Dorff's hermeneutical approach makes space for a significantly modern liberal skew. On the one hand, this reflects a commendable pragmatism on Dorff's part, as he pursues a theologically satisfying mode through which to balance competing commitments between parochial and universalist values. Yet from a secular viewpoint, it is unclear how to apply Dorff's central meta-ethic that "God owns the body" to practical issues in bioethics, since (at least as presented by Dorff) we can never be certain whether God's instructions (and by extension, contemporary religious practices) are righteous exactly as written (or as currently practiced), or outdated and deserving of reinterpretation, including through appropriate internal and external criticism. And nowhere is this more challenging than when it comes to the issue of the liberal values of bodily integrity and autonomy, and their interaction with the illiberal practice of NNC.

As previously noted, Dorff has not, to our knowledge, committed to any substantive treatment of this intersection, especially not in his writings on bioethics. He has, however, made some brief comments about religious circumcision in other texts, and we will use those to analyze his bioethics positions involving other issues that clearly bear on bodily integrity and autonomy. As we will show, this analysis reveals some confounding puzzles and contradictions.

Dorff on religious circumcision

While the topic of circumcision is largely absent from Dorff's works on Jewish bioethics, he has touched on the subject in other books[3] focused on religious practice and social action. A brief but

[3] For example, see the section on circumcision in *Jewish Choices, Jewish Voices: Body* (2008), edited by Dorff. While he does not take a position on the controversies himself, he includes opposing perspectives from two other Jewish authors. Feminist Harry Brod expresses regret over circumcising his son, while urologist and *mohel* Samuel Kunin defends newborn circumcision in his essay, "Circumcision: The Medical Imperative Accompanying the Religious Imperative" (Dorff and Newman 2008).

interesting commentary on the topic can be found in his book *The Way into Tikkun Olam (repairing the world)* (2007), where he describes circumcision as one of several Jewish "parental duties." As a part of the segment, Dorff touches on contemporary internal Jewish criticism of circumcision:

> In our own day, some Jews have questioned the wisdom of circumcising boys. The relevant medical associations have wavered on its medical advisability, recommending it during the last several decades of the twentieth century and now saying that its medical risks and benefits are roughly equivalent, making circumcision medically optional.
>
> The Jewish tradition, however, never advocated circumcision for medical reasons. Its purpose was instead religious. (Dorff 2007, p. 210)

To begin with, it is interesting that Dorff's immediate response to internal Jewish critique of circumcision is an appeal to non-religious medical associations (though he does not cite any in particular, we assume he means US groups; see Earp and Shaw 2017 for why), as if the permissibility of circumcision from a religious standpoint might hinge on medical advisability. It would be very interesting for Dorff to expand on this particular question, since the passage above seems to suggest that if relevant medical associations actively advised *against* NNC (as some European associations do; see Svoboda & Van Howe 2013; for a recent update, see Lempert et al. 2023), Dorff might be inclined to accept their recommendation. If that is the case, it would be a very important stance for medical associations to learn about, as some policymakers might currently assume that religious authorities such as Dorff would never accept such guidance.

However, the more important issue raised above is the meaning and application of the term "medically optional."

The portrayal of NNC as "medically optional," or as "medically neutral," by certain medical associations has grown in prominence almost uniquely in the US context, even more so than it had since Dorff published the statement above. The most significant source of this type of claim is currently that of the American Academy of Pediatrics 2012 policy (AAP 2012), which (though expired since 2017) supports "parental choice" around authorizing medically unnecessary newborn circumcision but stops short of recommending it, exactly as Dorff suggests. That policy, and that position more generally, rests on the idea that circumcision confers potential prophylactic health benefits (e.g., a reduction in the risk of some infections) so as to distinguish it from other practices which could be considered "merely" cosmetic. The AAP

policy even claims that “the preventative health benefits outweigh the risks of the procedure” (AAP 2012, 585). Many therefore originally initially interpreted the 2012 policy as a positive endorsement of the practice on medical grounds. Upon this reading of the term, “medically optional” may be taken as a recommendation without a mandate.

However, subsequent comments from the AAP policy’s authors have suggested that, first and foremost, the policy was intended to defend *parental* interests, with respect given predominantly to non-scientific values. The lead urologist for the policy taskforce, Dr. Andrew Freedman, explains in a 2016 clarification piece, these values include “religion, culture, aesthetic preference, familial identity, and personal experience,” and thus, “protecting this option [to circumcise newborns] was not an idle concern at a time when there are serious efforts in both the United States and Europe to ban the procedure outright” (Freedman 2016, p. 2). On this interpretation, the term “medically optional” appears to be better understood as a permission without recommendation.

One of us has critiqued the 2012 AAP policy on both scientific and ethical grounds in other writings (Earp 2017; Earp and Shaw 2017). So have various other scholars and scientists (Frisch et al. 2013; Svoboda & Van Howe 2013; see also Van Howe 2018). However, for the purposes of our present argument, it is significant that Dorff rightly understands religious circumcision as being performed for explicitly non-therapeutic reasons. We take it to mean that Dorff holds that under certain conditions, permanent surgical body modification can be justified without medical necessity, and without the consent of the person whose body will be impacted. This is the standard against which we will compare Dorff’s other positions on body modification below.

As to the non-medical rationales Dorff gives for circumcision (a subject which is an open matter of debate amongst other Jewish religious authorities), he writes:

> First, by literally inscribing a sign of God’s covenant with Israel on a man’s flesh, circumcision symbolizes that this covenant between God and the People Israel is immutable. Compare this to sealing a document in contrast to signing it: the seal changes the very surface of the paper, making it virtually impossible to change the document.
>
> Second, because circumcision changes the surface of a male’s generative organ, it symbolizes that this covenant is to last from generation to generation...

> Third, circumcision has implications for *tikkun olam* as well. In graphically tying Jews to the covenant, circumcision symbolizes Jews' commitment to Judaism's moral duties at least as much as to its rites. These include, of course, the duties [regarding] speech, the poor, captives, the sick, and one's parents and children. (Dorff 2007, p. 210-211)

In summary, Dorff produces the following religious rationales for circumcision: 1) an aesthetic rationale (which includes irreversibility as a virtue thus requiring a change in morphology rather than mere surface-level marks on the skin), 2) a familial and communal commitment and relation, and 3) an assignment of particularistic *morality,* for which Dorff appears to suggest that altering a child's "generative organ" will play a role.

While we are skeptical that any such reasons are sufficient to morally justify the performance of a non-therapeutic surgery on a young child's genitals (or for that matter on any person who is not capable of consenting), our skepticism is not the focus of this essay. Rather, what we are examining at present is whether Dorff can defend these arguments as permitting a *type* of practice he would endorse as a matter of principle, or if he instead views them as uniquely appropriate to newborn penile circumcision (perhaps especially as it is performed within traditional Judaism). We suspect the latter is more likely, as when we examine Dorff's other positions on non-therapeutic body modifications, it seems there are some values in conflict. These conflicts, we believe, stem from the fact that non-voluntary body modification isn't just an unusual or questionable practice by post-Enlightenment standards, but also by the standards of traditional Judaism (as understood and expounded upon by Dorff) as well.

An internal religious exception as much as an external one?

Although circumcision is viewed as foundational within most contemporary sects of Judaism, it is also exceptional in that the practice is not emblematic of (indeed, it is contrary to) Judaism's general religious ethic on body modification. Per the biblical commandment in Leviticus 19:28, the Torah generally prohibits cutting or marking the body, and Jewish scholars throughout history have consistently interpreted this as prohibiting practices like tattooing and other aesthetic body modification. Some even apply this precept as requiring the body to remain intact after death (Dorff 1998, 238) (which is why some Orthodox authorities prohibit autopsies and cadaver organ donation; see Rosner 1971). Therefore, the value of leaving the body unmodified is not only familiar to Jewish religious authorities and traditional communities, it is an accepted

value within those communities and is typically given significant normative weight by the associated authorities.

Thus (in line with his religious commitments), in most cases Rabbi Dorff presents the general Jewish ethic as *negatively* predisposed to non-therapeutic body modification, such as cosmetic surgery, tattoos, and piercings, even for consenting adults. For example, Dorff writes of tattoos that the rabbis of the Talmud believed "that putting permanent marks on one's skin violated the divine image in which we were all created" (Dorff 1998, p. 268).

However, in line with his apparent liberal leanings, we find that in almost all cases Dorff ultimately allows for (consenting, adult) individuals to choose for themselves. Despite his evident disapproval of such practices and endorsement of the religious prohibition as the ideal, he usually reasons that when a person modifies *themselves*, it can be tolerated religiously if their individual subjective values can be perceived as healthy in some meaningful way.

Consider the following two passages, in which Dorff assesses the ethics of cosmetic surgery and genital piercings:

> [Dorff on cosmetic surgery]
>
> Whereas therapeutic surgery seeks to reestablish what existed before and was damaged, cosmetic surgery done for aesthetic reasons seeks instead to improve the body a person inherited …
>
> … The proper response to such [aesthetic] pressure, however, is not to undergo the risks of cosmetic surgery to resemble others but rather to take nonsurgical steps to gain a better estimation of oneself…
>
> … In line with this view, one might think that cosmetic surgery for aesthetic reasons alone should be prohibited on theological, if not legal, grounds, for it is effectively saying that what God gave us is not good enough. As such, it is an act of ingratitude toward God, a lack of appropriate appreciation of the body God gave us. (Dorff 1998, p. 271)
>
> [Dorff on genital piercing]
>
> In general, Jewish law prohibits injuring ourselves or others. Body piercing escapes that prohibition because it is done in the name of adorning, rather than injuring, oneself. If the

> point of piercing the body is to add to one's beauty, though, one cannot but wonder how many people besides the person so adorned are supposed to see the pierced genital areas and thus appreciate them as a mark of beauty. If it is only the person whose body is pierced and that person's spouse, well and good. The Jewish value of modesty, though, demands that others should not be shown those areas of the body or the piercing or rings attached to them. (Dorff 1998, 270)

The first thing to note is that, as mentioned, Dorff generally frowns upon non-therapeutic body modification. As should be obvious, this principle or rule conflicts with NNC in a straightforward way, in that NNC (a non-therapeutic body modification) is religiously mandated while other types of body modifications are disallowed. But in a deeper sense, this also speaks to a conflicting *application of values*. Recall the positive religious rationales Dorff applied to NNC: an irreversible aesthetic transformation of the body, the binding of communal affiliation, and the motivation (indeed, commandment) to act in a certain moral manner. The high moral value Dorff places on NNC stands in contrast to the amorality or immorality he assigns to other body modifications. Circumcision is celebrated precisely for permanently altering the natural—or as some might prefer, "God-given"—body while other practices with similar physical outcomes demonstrate "a lack of appreciation of the body God gave us" (Dorff 1998, 271).

This suggests that the positive values applied to NNC are so exceptional that they are not even fit to serve as an *exemplary model* for other body modifications. Dorff derides movies and television for unduly influencing people to modify themselves. But if influencing (let alone "commanding") surgical body modification is a bad thing for a cultural movement to do, then what does that mean when religions (which are, after all, *also* cultural movements; Brusa and Barilan 2009) do the same? And how can Dorff's disvalue of surgical modification for the sake of "resembling others" be reconciled with NNC: a practice which is intended to mark male membership in the group collective, such that all are expected—indeed, most are forced—to "resemble" each other in a very specific way?

Dorff's comments on genital piercing are especially curious in this regard, as they seem to demonstrate consternation towards others who value or perform alterations of the *genitals* in particular. This is surprising since it could reasonably be assumed that if one regards a non-therapeutic genital modification as foundational to one's own belief system, one would be at least sympathetic toward others who also practice genital modifications, even if their preferred modification is different than one's own. However, in the passage above, Dorff appears entirely

perplexed by the idea that anyone would want to interfere with their intact or “God-given” genitalia.

In other words, Dorff does not appear to entertain the notion that one who pierces their own genitals (let alone some else's) might *also* be invested in some significant spiritual or communal objectives, such as marking their “generative organ” for the purposes of intergenerational continuity, or, perhaps, out of fealty to someone or something the individual believes owns their body—remarks that plausibly apply, not only to Jewish penile circumcision, but also to the Islamic practice of “ritual pricking” as we will soon examine. Indeed, he goes so far as to claim that religious virtue “demands” that such behavior be kept as private as possible, clearly interpreting genitals as an intimate domain (a traditional as well as a contemporary concern; see Bettcher 2023; Buckler, Bruce, and Earp 2023), rather than as a part of the body to be marked or displayed in the context of public (e.g., community-building) behavior.

Now, it may be argued that since Dorff ultimately condones or tolerates most common body modification practices (i.e., he doesn’t advocate their prohibition), there is no real conflict with NNC.[4] But it must be understood that basis for Dorf’s begrudging attitude of permissibility toward these practices is his respect for *individual subjectivity and autonomy*. As he writes: “in the end such procedures are permitted in the name of augmenting a person’s own sense of self-worth and, hence, mental health” (Dorff 1998, 272). Personal choice is what ultimately overrides the religious prohibition for Dorff. In other words, when it really matters to his position in the public square, Dorff is—effectively, if not very enthusiastically—a supporter of bodily autonomy.

Yet in all such cases in his works on medical ethics, Dorff approaches the ethics of aesthetic body modification solely from the angle of permissibility for a competent decision maker to alter their *own* body. Nowhere does Dorff apply this reasoning to the notion that an adult may consider using their subjective aesthetics or conceptions of mental health—much less, their religious beliefs—to justify tattooing or surgically altering *another* person’s body for non-therapeutic reasons without their consent.

Thus, as stated earlier, circumcision is an exceptional practice, even within Dorff’s religious framework.

[4] Other religious authorities have noted that even Orthodox Jewish law can makes space for certain body modifications and piercings (such as ear piercing and cosmetic procedures such as rhinoplasty) if they have become commonplace in society. For a discussion, see filmmaker Eliyahu Ungar-Sargon in conversation with Rabbi Asher Lopatin (Ungar-Sargon 2018, 19:10). But there is no indication that these may be performed non-consensually, and abstention from body modification remains the normative value, while modifications require special justification.

Now, this is a simple enough inconsistency to reconcile internally, where the primary impetus to follow God's commandments may be taken to justify exceptional practices; but it is a more difficult matter to explain when it comes to the public square. It is hard for one to defend a practice to outsiders when in most other similar cases they would strongly disapprove. Dorff may point to Genesis 17 and other sources to explain Jewish circumcision. However, if we asked Dorff for the Judaism-informed bioethical position on the permissibility of a practice involving cutting or excision of *other* healthy genital tissues, or the amputation of (part of) some *other* healthy bodily organ, we expect that his answer would follow much as it does in the *Oxford Handbook*: that because neither our bodies—nor our children's bodies–-are our own, we should not have the "right" to alter or harm them.

Now, consider what this would mean for our hypothetical taskforce on the child's RBI. It seems to us that because of circumcision's exceptionality in Judaism, the Jewish principle of GOTB as presented by Dorf would not be helpful to our analysis, as it is used to explain both the exception *and* the rule.

Yet this leaves us with a major remaining question with practical consequences for the debate: How does Dorff approach *other* religious genital cutting practices? If Dorff may claim that it is permissible to cut a child's body based on the belief that God has commanded it, or favors it, would he be willing to accept this very same argument when presented by a follower of a different religion? We explore this question in the following section.

Does Dorff's position obligate him to oppose genital cutting or alteration if pursued by other religions?

So, what about the followers of other religions? Should they also be permitted to cut their children's genitals? This question is not merely theoretical, as there are several other types of religiously motivated child genital cutting practices globally. However, many Western states have enacted legislation prohibiting other forms of medically unnecessary child genital cutting, including all types affecting children with female-typical genitalia, regardless of the religious beliefs or intent of the parents/community, and no matter how minor or "symbolic" the given procedure may be (see, e.g., the US federal STOP FGM Act of 2020, passed into law as Public Law No: 116-309 in 2021; for context, see Earp 2020).

Indeed, a recent a string of high-profile legal cases in the USA and Australia have highlighted this issue, with controversy centering on the Dawoodi Bohra community (a Shi'a Muslim sect with roots in South Asia), whose female genital cutting practice is allegedly constrained to a mitigated "symbolic" ritual nick, or prick (Rogers 2019; Bootwala 2019a, 2019b,

2019c). Members of the Bohra community who support the practice have argued that there is a double standard at play, since penile circumcision—which they also practice—is currently permitted in Western countries, even when medically unnecessary and non-consensual, yet is substantially more physically invasive than their female variant (Zakir 2016; DBWRF 2018).

While Dorff has not commented on these controversies,[5] our reading of his positions leaves us unsure what he would recommend, since we could equally imagine him reaching one of two diametrically opposed positions:

1) Dorff could *condemn* other types of child genital cutting, including by supporting protections for children, since his own religious code ordinarily compels him to do so.

2) He could *condone* other forms of child genital cutting, since he usually endorses religious freedom as a civic right, and equal treatment of different religious, even if his own internal religious code would prohibit other types of religious practice.

While it is difficult to speculate which of these positions Dorff would adopt, we believe that either one risks undermining the coherence of his general philosophy. No matter which course of argument he prefers, he will have to betray a major value. Allowing other types of child genital cutting rituals would offend his own God's commands; yet prohibiting them would seem to offend the value of religious pluralism and equal respect for different religions.

Consider position #1. As established earlier, Dorff claims that a fundamental conviction in Judaism is that the body belongs to God (GOTB) and we are therefore forbidden from making permanent, non-therapeutic changes to the body or otherwise causing the body "unnecessary" harm. This precept extends to a seemingly generalized prohibition on aesthetic body modification as described above. As noted, Dorff allows for certain exceptions to this prohibition in the case of consenting adults choosing for themselves, but in the case of *non-consensual* genital cutting it cannot be attached to uniquely personal reasons. It would therefore seem, at least upon a first look, that a position *opposing* child genital cutting (CGC) would be the most natural outgrowth of Dorff's usual arguments.

[5] Dorff provides the following explanation as to why Judaism itself does not include a female genital cutting variant to accompany ritual circumcision of the penis: "The same symbolism was not needed in removing a girl's clitoris, presumably because from ancient times to the modern era the man was the head of the house, and therefore making men part of the covenant throughout all generations immediately made all the female members of their families part of the covenant as well" (Dorff 2007, p. 211).

In fact, there is another major reason why Dorff could be expected to oppose CGC, which we have not yet touched upon directly, which is the potential of such practices to be experienced as acts of sexual trespass or violence. Many self-described victims of religiously motivated genital cutting compare the experience to sexual assault—or other violation of sexual boundaries—including in situations where the cutting itself is supposedly 'minor' (at least relatively so), as with ritual pricking (Kumar 2018). Consider the experience of Saleha Paatwala, a member of the Dawoodi Bohra community, who was taken by her grandmother, when she was 7 years old, to undergo the prescribed ritual for girls in her community. Referring to the unfamiliar woman who carried out the procedure at her grandmother's behest, she states:

> She asked me to lie down and, uh, this very thought gave me goosebumps all over my body. [This] woman started pulling down my underwear. And that whole idea ... it was really scary. She took it out and now she spread my legs, grabbed the blade, and cut something between my legs. It was definitely painful. [But] it was more embarrassing because a lady whom I did not know saw my [private] area at that point of time, and she did not see but she also cut ... (Paatwala 2019).

When considered in this light, the stakes of ethical debates around CGC move beyond an assessment of the practices considered as body *modifications* and expand to include an assessment of them as *actions* taken by adults toward children's genitalia: actions whose symbolic significance—as interpreted by the child, whether in the moment, or upon later reflection—cannot be fixed by the adults in advance (Buckler, Bruce, and Earp 2023). In other words, as one us has argued elsewhere, concerns about the bodily *state* produced by a non-consensual genital procedure (e.g., whether this state reflects, or amounts to, a net benefit or harm) miss the more basic consideration that such a procedure involves, necessarily and in the first instance, intrusion into the child's sexual embodiment: that is, a part of the body commonly considered to be especially intimate or private, such that a person can be violated by "mere" non-consensual touching of, or even looking at, their genitalia (see Box 1), at least outside of certain rare medical emergencies (Buckler 2023).

Considered in the abstract, this is a point with which, we expect, Rabbi Dorff would be inclined to strongly agree. After all, he, too, argues that the genitals should be seen as a distinctively private and protected site, including from a Jewish religious perspective. See, in particular, his fascinating *responsum* on "family violence" (Dorff 1995). On the impermissibility of interfering with another person's genitals (even for an otherwise righteous, explicitly non-sexual cause), Dorff writes,

> The Torah makes this exceedingly clear: 'If two men get into a fight with each other, and the wife of one comes up to save her husband from his antagonist and puts out her hand and seizes him by his genitals, you shall cut off her hand; show no pity [Deut. 25:11-12]:' Despite the special justification the woman had for shaming her husband's assailant, the Torah demands drastic steps in retribution for the degradation she caused. (Dorff 1995, 790)

Dorff thus concludes that, “We are humiliated when we are sexually abused—even just touched in our private parts against our will—for we feel that our sense of self has been invaded, that our honor has been compromised in the most fundamental way possible” (Dorff 1995, 790). And Dorff is especially protective of children. He writes: “Indeed, in light of the extensive damage it causes to the future ability of the child to cope with life, without too much exaggeration I would say that, in the case of children, sexual abuse is akin to murder” (Dorff 1995, 791).

What does this imply for Dorff’s likely position on ritual pricking in the Bohra community? We suspect it would be challenging for him to defend an accommodation of the practice, whether by means of a legal excuse or other type of exception (see Merkel and Putzke 2013). At least, it would be a struggle if he tried simultaneously to defend his more general ethic that God owns the body; that it’s his duty to try to prevent harm to God’s property; and that one should obey biblical commandments like Leviticus 19:28; all while remaining consistent with his own rabbinic rulings on sexual health, violence, and personal boundaries, including within the family.

Box 1. Can medically unnecessary genital procedures, even if performed with good intentions, be experienced as a violation of sexual boundaries? The question of timing.

Whether a medically unnecessary genital procedure is experienced or interpreted by the individual as a type of trespass on their sexual boundaries could depend, some might argue, on the timing. For example, one could argue that ritual pricking, insfofar as it is carried out while the child is fully conscious and aware of her genitalia as private anatomy, there is a heightened risk that the act will be experienced or interpreted as a sexual violation, at least when compared to penile circumcision as performed in infancy, that is, before the child is likely forming any consciously-retrievable memories, which is when it is traditionally practiced within Judaism.

However, first, it must be noted that boys in many communities, including some Muslim communities, are also often circumcised under coercive conditions while conscious and sexually self-aware: that is, as older children, rather than in infancy (Lee 2006; WHO 2008; Anwer et al. 2017; Rizalar et al. 2017; Yavuz et al. 2024). Moreover, there is evidence—from a range of cultural settings—that an unknown proportion of men (and some transgender women) who underwent NNC in in infancy or childhood do feel a sense of sexual violation or other significant

distress in relation to the procedure, sometimes upon later reflection (Bossio & Pukall 2018; Remennick 2020; Hammond et al. 2023; Uberoi et al., 2023; Tao 2024).

With respect to Jewish practice, although the newborn period is when the vast majority of circumcisions are performed, circumcision of older boys does sometimes occur, for example, in the case of adolescent males who convert to Judaism (see also Remennick 2020 for a different example). There is also the Jewish ritual practice of *hatafat dam brit*—which, similar to the Bohra practice for girls—involves pricking or piercing the sexual organ to draw a drop of blood. *Hatafat dam brit* is more commonly performed on older children (typically, already-circumcised males whose circumcisions were considered ritually invalid); and it is also experienced as a sexual boundary violation by some who undergo it. As one man recounted (quoted in Buckler 2024; see that paper for further examples and discussion),

> I was … old enough to be worried when Dr. Greenbaum said, "Joshua, please lower your pants." I began bawling as I inched my tightie-whities down my thighs, revealing my teenage [penis]. The doctor grasped my offending member [and] pricked my bunched skin and squeezed until a blood dollop oozed from the minute hole … On the rabbi's command, I stripped and stepped into the mikveh [ritual bath] … Instead of pride, I felt shame: my rabbi had seen me naked before my eighth-grade girlfriend.

This is not to say that ritual genital procedures performed on infants or newborns have no implications for sexual boundaries. For example, suppose the Bohra community—like various other communities that traditionally practice ritual female, as well as male, genital cutting—decided to change the timing of the practice to infancy (in some communities, this is a response to the practice becoming criminalized; see Earp et al. 2024), perhaps based on the reasoning that "she won't remember it." We think it is unlikely this would mollify those who regard the practice as a serious trespass. This is because, first, in the case of unambiguous violations (e.g., sexual molestation), it is typically considered morally *worse*, not better, if the victim is unconscious or a very small child, irrespective of whether the child later learns about what happened.

And second, as we and others have suggested elsewhere, while it is true that "very young children, including infants, do not (yet) [have] a sense of their genitalia as 'intimate' anatomy … most will come to acquire such [a] sense. If they learn, therefore, that prior to their ability to exercise [an] essential boundary-setting right, their 'intimate' anatomy was *already* cut or altered for reasons other than medical necessity, they may reasonably come to conclude that (what should have been) an exceptionally personal choice about their sexual embodiment has been usurped" (BCBI 2024, p. 8).

On the other hand, consider the potential for taking position #2, and the challenges this might raise for Dorff's approach to bioethics: namely, arguing for the *permissibility* of other types of CGC. Dorff ostensibly supports total equality in civil law, and claims there is a Jewish religious basis for this position, as he writes, "[In] thirty-six passages, by the rabbis' count, the Bible boldly proclaims equality in law between Jew and alien" (Dorff 1998, 272). Thus, in most cases we would expect Dorff to support religious freedoms equally and without discrimination within a pluralistic society, including in relation to practices which are prohibited by the particularistic requirements of Judaism. And, as we've seen, Dorff is well-equipped to navigate around many biblical prohibitions where they conflict with contemporary values. Thus, it is not inconceivable that Dorff could come up with a flexible approach (as he does with personal cosmetic surgery) to other religious practices out of respect for value pluralism.[6]

Setting aside the aforementioned potential for these practices to be experienced as violations of sexual boundaries, the relatively minimal invasiveness of certain types of female genital cutting (FGC) countervails oft-raised concerns over detrimental physical changes, which is a common knee-jerk objection to such cutting (see generally, Catania, et al, 2007). Some of these practices are indeed so "minor" that they can hardly be said to modify the body at all, and it is therefore argued by some of the groups who practice such rites that they are not harmful to the body or its natural function (see above). In fact, such a position was once supported by the AAP, when it attempted a policy aimed at permitting ritual pricking or a similar procedure in order to offer a "medically optional" practice (or rather, "a medicalized option") to relevant religious communities (AAP 2010; for discussion, see Van Howe 2011).[7]

Yet, regardless of the potential for these practices to be mitigated in their physical impact, we also point out that Dorff has placed religious value in *significant* morphological change in the body for penile circumcision, a procedure that removes, in the form that currently predominates within Jewish practice, roughly one-third to one-half of the motile skin system of the penis (Taylor et al. 1996; see also Fahmy 2020; Cepeda-Emiliani et al. 2023). (Recall how Dorff explained that "sealing a document" would be greater than just "signing it.") So, it seems reasonable that Dorff could also potentially support some acceptable *range* of genital modifications (ranging from the most superficial to perhaps some types of relatively minor modifications, for example, of the inner labia), if for no other purpose than to stay consistent in his reasoning.

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[6] Indeed, such a position has been forcefully defended by the anthropologist Richard Shweder (Shweder 2009; 2022).

[7] Paradoxically, this position struggles to gain traction because of the very fact that pricking/nick is *so* minor it cannot be attached to health benefits *á la* circumcision. See Buckler (2024).

However, the taking of either of these positions—condemn or condone—would have challenging implications for Dorff's central ethic. Per Dorff's commitment to equality in civic law, it could be viewed as hypocritical for him to oppose FGC in its most minor forms, as whatever protections can be offered to girls should apply equally to boys (and perhaps also to children with intersex traits, whether assigned female or male; see BCBI 2024; Delaet, et al 2024). Yet taking the second position—*condoning other forms of CGC*—might be perceived as equally problematic for Dorff, particularly as it relates to the question of this volume: namely, Should religion play a role in bioethics?

This is because Dorff cites God's ownership of the body as a fundamental religious conviction (GOTB), guiding both his personal ethic and his distinctively religious approach to bioethics. When, therefore, is it relevant to bioethical debate? As we have seen, it appears that, in practice, Dorff's religious conviction most often yields to contemporary secular arguments based on liberal values, autonomy-promoting stances on the body, and the requirements of public reason. It may be wondered, then, what the principle of GOTB really adds to bioethical analysis? After all, insofar as the principle seems to conflict with secular liberal conclusions, God, for the most part, does not seem to get his way.[8]

Although there are no easy answers here, we believe that such questions must be raised. With Dorff, we agree that even the most contentious issues in bioethics require vigorous debate—including both religious and secular perspectives—in the public square.

Conclusion: an invitation to meet in the public square

As stated at the beginning of this chapter, we welcome religious engagement in the field of bioethics as much as any other viewpoint, so long as the same argumentative rules apply across the board. Accordingly, we respect Rabbi Elliot Dorff's immense efforts in the field of ethics, and we have focused on his work here precisely because it can be so fruitfully put into conversation with that of others. As Dorff writes, "Although God may be omniscient, no human being is, so the best way for us to attain truth, goodness, and wisdom is to pool our intellectual and moral resources in serious conversation" (Newman and Dorff 2024, 100).

We would therefore invite Rabbi Dorff to weigh in on the topic of religious NNC of the penis, and its relation to the child's RBI, particularly when considering other forms of child genital cutting and modification done for religious reasons, such as ritual 'pricking' of the vulva

[8] We are using "his" self-consciously here; it is how God is typically gendered in Jewish (and Christian, etc.) scripture.

as performed in some Muslim communities. Given the significance of NNC to traditional Judaism, and the increasing degree of controversy surrounding the practice, the time for public discussion including both secular and religious viewpoints on the topic has surely come. A scholar of Dorff's pedigree should be expected to contribute to any such discussion.

When we consider arguments from any philosopher—religious or not—we expect their positions to apply consistently, and we expect them to engage forthrightly in difficult challenges to their convictions, especially when they apply to unusual practices. This is not to say exceptions to general principles can never be made when weighing competing values, but when a philosopher proposes an exception (and hopes to convince us of its validity), they should be able to explain what precise conditions must be met for the exception to apply. Consequently, wherever else those conditions are met, the exception should also be available to others equally (Shweder 2017; Ahmadu 2017; Ahmadu & Kamau 2022).

For these reasons, we believe the issue of child genital cutting is a crucial test cast for Dorff, and for religious bioethics more generally. We invite him to share his views on this matter.

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